

Haven Paediatric Centre

Proposal for operational turnaround, culture, and growth

Building the culture, standards of work, and incentives that make good care self-sustaining. Then reengineering the operations and unlocking growth.

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1. Why we are having this conversation

Haven Paediatric Centre was built to raise the standard of paediatric care in Nigeria. Fifteen months in, it is funding its own salaries, holding a steady patient base, and earning a real reputation in Ikeja. That is a genuine achievement for a young facility.

Leadership is now doing what the strongest operators do: looking hard at the systems beneath the surface before growth outpaces them. As Haven scales into more complex care, including neonatal intensive care, its clinical governance, its standards of work, and its financial discipline need to be deliberate rather than assumed. Putting that foundation in place now, while the facility is still young, is far easier than retrofitting it later.

This proposal sets out how Consult for Africa would help, the shape of my own involvement, and exactly what it costs, at full transparency.

2. The honest diagnosis: culture is the multiplier

It is tempting to treat operational improvement as a set of process fixes. In practice they rarely hold unless the culture, the standards of work, and the incentives beneath them are built first. Strong routines are what make safety and efficiency automatic, rather than dependent on individual diligence.

The same theme shows up across the business:

- Consistent, shift-level clinical routines are the backbone of reliable care, and formalising them is foundational as the facility takes on more complex cases.
- Margins are thin despite reasonable revenue, because there is no ownership culture pushing efficiency and yield.
- Two items already on the board's own decision list, the staff commission structure and the JDS and KPIs, are exactly the incentive and accountability tools a strong culture runs on. The report is already pointing at the answer.

So the work is not to patch processes. It is to build the culture, the standards of work, and the incentives that make good processes self-sustaining, then reengineer the processes themselves so the culture has something solid to run on.

3. What the last management report already tells us

The cash problem is a working capital problem, not a profit problem.

Leadership feels there is “barely anything left” after salaries. The report shows why: roughly N4.2M sits in HMO receivables (Leadway and NEM alone are the whole balance), close to a full period's revenue, and a further N2.77M sits in pharmacy stock. That is around N7M of working capital locked up. The money is not missing. It is on the shelf and in the HMO ledgers.

NICU is the growth engine, and it rewards getting the basics right first.

At a N3M deposit per admission against three beds, NICU is the single highest-yield lever in the building. Scaling those admissions durably depends on putting the governance, standards, and team readiness in place first. Get that right, and the growth compounds. That is the through-line of the whole engagement.



The reporting layer is immature, and that is itself a finding.

The visit-type counts do not reconcile to total encounters, the receivables table does not foot, and a 201 percent pharmacy “profit” is actually a markup, not a margin. None of this is incompetence. It is the absence of a senior operator. You cannot manage what you cannot measure reliably.

4. Proposed involvement and engagement structure

A single board-led turnaround with five workstreams. The first four are the core project. The fifth is an optional, ongoing oversight retainer leadership can opt into.

1. Diagnostic audit (4 weeks)

A detailed audit across clinical governance, operations and SOP adherence, finance and working capital, procurement and inventory, HMO economics, staffing, and the reporting layer. Two quick wins run from week one: a checklist-governed crash cart, and an immediate recovery push on the Leadway and NEM receivables.

2. Culture, incentives, and clinical standards of work

The spine of the engagement. Establish the shift-level clinical routines, the nursing standards of work, and the safety-huddle cadence. Redesign incentives: the commission structure and the JDS and KPIs already awaiting board approval, aligned to quality and ownership rather than activity alone.

3. Process reengineering and operations

Move procurement and inventory to a vendor-managed model. My specific recommendation is to engage Medbury Pharma for vendor-managed inventory, which keeps critical stock dependable and lifts pharmacy margin at the same time. Build the receivables recovery process and a reliable management reporting layer.

4. Revenue and growth optimisation

Internal growth (NICU activation, pricing review, pharmacy attach, HMO yield) and external growth (corporate tie-ups, school partnerships such as Toddler Town, and a referral pipeline).

5. Board-level management oversight (optional retainer)

Up to two days per month of partner-level oversight from me or a delegate, continuing after the core project. The ongoing governance layer, offered as an opt-in.

5. The senior operations leader

One thing I advised early still holds, and leadership is now ready for it: Haven needs a senior, experienced operations leader running the facility day to day. The audit defines that role and its KPIs first, so the hire lands into clarity rather than chaos. Consult for Africa can source and vet that person through CadreHealth, our workforce platform, including diaspora and senior operator profiles. Define the role during the engagement, then recruit into it.



6. Timeline

Phase	Timing	Focus
Stabilise	Weeks 1 to 2	Crash-cart standard and checklist, receivables recovery push
Diagnostic audit	Weeks 1 to 4	Full audit across all functions
Culture and process build	Months 2 to 4	Standards of work, incentives, procurement, reporting
Growth	Months 3 to 6	NICU activation, pricing, corporate and HMO tie-ups
Oversight	Ongoing (optional)	Board-level management retainer

7. Commercials, in full transparency

We price every workstream at standard Consult for Africa rates so the board sees the real value, then show a single partner discount line, then the net. Nothing is hidden in the rate. Given our longstanding relationship, and that I have been invited to Haven's board, the facility receives a 40 percent discount on every line.

Workstream	Standard rate	Partner rate (40% off)
1. Diagnostic audit (4 weeks)	N3,500,000	N2,100,000
2. Culture, incentives and standards	N5,000,000	N3,000,000
3. Process reengineering and operations	N4,000,000	N2,400,000
4. Revenue and growth optimisation	N4,500,000	N2,700,000
Core project total	N17,000,000	N10,200,000

The discount is a N6,800,000 concession, shown in full rather than buried in a lower headline rate.

Optional ongoing oversight (Workstream 5): standard N1,000,000 per month, partner rate **N600,000 per month**, for up to two days per month of partner-level oversight. Priced to senior-partner value and offered as an opt-in alongside the core project.

Payment schedule: a mobilisation fee on signing, then the balance spread in equal monthly instalments, so cost tracks delivery and is comfortably covered by the receivables and margin the early work unlocks.

Payment	When	Amount
Mobilisation (diagnostic audit)	On signing	N2,100,000
Monthly instalment (x5)	Months 1 to 5	N1,620,000 each
Core project total		N10,200,000



The optional oversight retainer, if taken up, is billed monthly and separately from the schedule above.

8. A note on governance

I have been invited to Haven's board, and Consult for Africa would be a paid partner to the company. That overlap is common and entirely workable, and I am raising it precisely because formalising your governance is part of this work. The pricing and the discount are disclosed in full to all owners, and the engagement is structured as clean fixed fees plus an optional retainer, with no success fee tied to outcomes I would oversee. Transparency here is part of the standard we are setting for the facility.

9. What good looks like in six months

- A safety culture in which the crash cart, and every critical routine, is checked every shift, by habit, not by reminder.
- The N4.2M in receivables recovered and a working-capital discipline that keeps cash moving.
- A vendor-managed inventory model that ends stockouts and lifts pharmacy margin.
- NICU admissions growing safely against a governance standard the team trusts.
- A senior operations leader in post, running a facility that measures itself honestly.
- Incentives that reward quality and ownership, so the culture sustains itself after we step back.

10. Next steps

If the board is content with the direction, the first step is to commission the diagnostic audit. From there, the audit findings shape the detail of the culture, process and growth work, and the timeline firms up around real data.

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